

Sleepgenic

The Problem We're Solving

Plain-Language Briefing

THE SCALE OF THE PROBLEM

Sleep disorders are one of the most common medical conditions in America — and one of the most ignored. Roughly 50 to 70 million Americans have an active sleep disorder. Of those, about 30 million have been formally diagnosed with chronic insomnia — meaning a doctor has confirmed they have a real clinical condition, not just a bad habit.

That's not a niche market. **That's the size of California and Texas combined.**

On top of that, 1 in 3 American adults doesn't regularly get the recommended 7 hours of sleep. And two-thirds of adults experience insomnia symptoms at some point — most of them never getting clinical help.

WHY NOBODY IS GETTING TREATED

Here's where it gets striking. Despite 30 million diagnosed cases, 47 to 67% of people with insomnia never seek medical attention for it. Of those who do seek help, most get lifestyle advice from a primary care doctor — “try cutting caffeine, keep a consistent bedtime” — not actual evidence-based treatment.

The gold standard treatment, per the American Academy of Sleep Medicine, is **CBT-I (Cognitive Behavioral Therapy for Insomnia)**. It's a structured behavioral protocol that rewires the mental and physical patterns driving poor sleep. It works better than sleeping pills long-term, with no dependency risk.

The problem: fewer than 500 certified CBT-I therapists exist in the entire United States. Waitlists routinely exceed one year.

THE CRITICAL DISTINCTION — THIS IS NOT OUR BOTTLENECK

That 500 number sounds alarming but it's important to understand exactly what it means.

CBT-I certification is a formal multi-year credential in behavioral sleep medicine — similar to a subspecialty fellowship. It is not a requirement to prescribe sleep medications or to follow a CBT-I protocol in clinical practice. Licensed physicians and nurse practitioners can do both without that certification.

Sleepgenic's clinical model uses a partner provider network — staffed by licensed MDs and NPs — who prescribe evidence-based sleep treatments and follow CBT-I protocols as standard care. We route around the certified therapist bottleneck entirely.

The <500 stat doesn't constrain our supply. It explains why the access gap exists in the first place — and why no one has solved it yet.

THE RATIO THAT SAYS EVERYTHING

- **Certified CBT-I therapists in the U.S.:** fewer than 500
- **Americans with diagnosed chronic insomnia:** ~30 million
- **Ratio:** 60,000 patients per certified provider

That is not a healthcare system. That is a structural gap masquerading as one.

WHAT SLEEPGENIC DOES

Sleepgenic is the access layer that doesn't exist yet. A patient fills out a 10-question biometric intake — pulling from data they already have on their Garmin, Whoop, or Apple Watch — pays \$149–179/month, and receives a licensed provider review within 24 hours.

Treatment starts with CBT-I protocol. Medication is added where clinically indicated — trazodone off-label, or an FDA-approved orexin antagonist like Quviviq.

No waitlist. No specialist referral. No insurance maze. Cash-pay, async, and built for the patient who already knows something is wrong — they just have nowhere to go.

THE REGULATORY ADVANTAGE

Unlike GLP-1 weight loss telehealth — which ran into compounded drug risk and FDA gray areas — sleep medicine has a clean regulatory profile. CBT-I is a behavioral protocol. Trazodone is a decades-old generic. Orexin antagonists are FDA-approved branded drugs. No compounding exposure. No regulatory overhang.

THE BOTTOM LINE

Thirty million diagnosed patients. Fewer than 500 specialists. A 60,000-to-1 patient-to-provider ratio. A gold-standard treatment that requires no specialty certification to deliver. A cash-pay model proven at scale by GLP-1 telehealth. And a patient who arrives already holding their own biometric data.

The market exists. The demand is documented. The regulatory path is clean. The infrastructure is available. **Sleepgenic is the brand that makes it accessible.**